



Laboratory Investigation Report

Patient Name	: Sonu Yadav	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 2 M 25 D /F	OP/IP No	: OP/101693
Max ID/Mobile	: 372462/8752005200	Collection Date/Time	: 13/Sep/2021 03:41PM
Lab ID	: 1266092100294	Receiving Date	: 13/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 14/Sep/2021
Passport No.	:		

Serology Elisa

IgE (Immunoglobulin-E)*, Serum

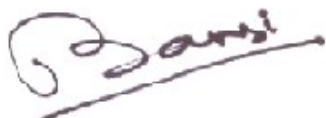
FEIA

Date	13/Sep/2021 03:41PM	Unit	Bio Ref Interval
Immunoglobulin-IgE	688	kUA/L	0-64

Comment Total IgE is an in vitro test system for the quantitative measurement of circulating total IgE in human serum or plasma. It is intended for in vitro diagnostic use as an aid in the clinical diagnosis of IgE mediated allergic disorders in conjunction with other clinical findings, and is to be used in clinical laboratories. A definite clinical diagnosis should not be made as a result of single test only, but should be made by taking into account clinical history and other laboratory findings.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Bansidhar Tarai, M.D.
Associate Director
Microbiology & Molecular Diagnostics



Dr. Poornima Sen, M.D.
Consultant - Microbiology



Dr. Madhuri Somani, M.D. , DNB
Consultant - Microbiology





Laboratory Investigation Report

Patient Name	: Sonu Yadav	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 2 M 25 D /F	OP/IP No	: OP/101693
Max ID/Mobile	: 372462/8752005200	Collection Date/Time	: 13/Sep/2021 03:41PM
Lab ID	: 1266092100294	Receiving Date	: 13/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 14/Sep/2021
Passport No.	:		

Clinical Biochemistry

Test Name	Result	Unit	Bio Ref Interval
IgG (Immunoglobulin-G)*, Serum			
Immunoglobulin IgG Immunoturbidimetric	1181.42	mg/dL	700 - 1600

Interpretation

Polyclonal increase seen in autoimmune diseases (systemic lupus erythematosus, rheumatoid arthritis, Sjögren's syndrome), sarcoidosis, chronic liver disease, some parasitic diseases and chronic or recurrent infections.

Monoclonal gammopathies seen in IgG type multiple myeloma, lymphomas, leukemia, and other malignancies.

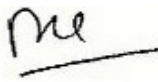
Hypogammaglobulinemias, IgG deficiency may be genetic as in severe combined immunodeficiency or acquired as in AIDS. A decrease in IgG also occurs as a result of thermal burns, nephrotic syndrome, protein losing enteropathies and non-IgG myelomas.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology





Laboratory Investigation Report

Patient Name	: Sonu Yadav	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 2 M 25 D /F	OP/IP No	: OP/101693
Max ID/Mobile	: 372462/8752005200	Collection Date/Time	: 13/Sep/2021 03:39PM
Lab ID	: 1266092100294	Receiving Date	: 13/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 16/Sep/2021
Passport No.	:		

Outsourced

Test Name	Result	Unit	Bio Ref Interval
-----------	--------	------	------------------

MTHFR Gene Mutation PCR (L)*, EDTA

Real Time PCR

MTHFR (Methyl Tetrahydrofolate Reductase) GENE MUTATION

C677T	Heterozygous mutation detected
A1298C	Heterozygous mutation detected

Ref Range

Result	Comments
Homozygous mutation detected	Both copies of the gene carry mutation.
Heterozygous mutation detected	One copy of the gene carries mutation.
Not Detected	Mutation not detected.

Note

1. This assay detects the mutation C677T and A1298C in MTHFR.
2. This is an in-house developed assay
3. Test conducted on Whole blood
4. Genetic counseling available.

Comments

Homozygous mutation for MTHFR (Methyltetrahydrofolate reductase) is associated with hyperhomocysteinemia which is an independent risk factor for Stroke, Myocardial infarction, Peripheral arterial disease and venous thrombosis. Indian studies suggest that heterozygosity for MTHFR C 677T is also associated with elevated homocysteine levels. MTHFR C677T or A1298C carriers are not at increased risk for thrombosis in the absence of hyperhomocysteinemia. Homozygous MTHFR C 677T or A1298C carriers are at increased risk for hyperhomocysteinemia if they become deficient in vitamins B 6, B12 or folic acid. Hyperhomocysteinemia is a relatively weak risk factor for both venous thromboembolism and arterial thrombosis. According to ACMG guidelines there is growing evidence that MTHFR polymorphism testing has minimal clinical utility and, therefore should not be ordered as a part of a routine evaluation for thrombophilia.



Page 3 of 5

SIN No: B2B899479, Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017
 Bool Max Lab - A Division of Max Healthcare Institute Ltd. Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002,
 8752 Max Super Specialty Hospital, Saket (West Block)

1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.
 The authenticity of the report can be verified by scanning the QR Code on top of the page

CIN No.: L72200MH2001PLC322854

www.maxlab.co.in

Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.

**Laboratory Investigation Report**

Patient Name	: Sonu Yadav	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 2 M 25 D /F	OP/IP No	: OP/101693
Max ID/Mobile	: 372462/8752005200	Collection Date/Time	: 13/Sep/2021 03:39PM
Lab ID	: 1266092100294	Receiving Date	: 13/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 16/Sep/2021
Passport No.	:		

Test Name	Outsourced Result	Unit	Bio Ref Interval
-----------	-------------------	------	------------------

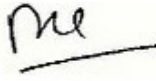
Note : Outsourced Test

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



Page 4 of 5

SIN No: B2B899479, Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017
Boo Max Lab - A Division of Max Healthcare Institute Ltd. Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002,
8752 Max Super Specialty Hospital, Saket (West Block)
1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.
The authenticity of the report can be verified by scanning the QR Code on top of the page

CIN No.: L72200MH2001PLC322854

www.maxlab.co.in

Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.

**Laboratory Investigation Report**

Patient Name	: Sonu Yadav	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 2 M 25 D /F	OP/IP No	: OP/101693/
Max ID/Mobile	: 372462/8752005200	Collection Date/Time	: 13/Sep/2021 03:37PM
Lab ID	: 1266092100294	Receiving Date	: 13/Sep/2021
Ref Doctor	: Dr. Richika Sahay Shukla	Reporting Date	: 14/Sep/2021
Passport No.	:		

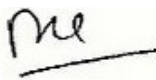
Hematology Special**Natural Killer Cells, Flow Cytometry, EDTA****Flow Cytometry Number****IM 154/2021**

Total Leucocytes	8500	cells/ μ l
Lymphocyte percentage	28.4	%
NK cells percentage:	7.21	% of gated lymphocytes
NK cells absolute count	174.11	cell/ μ l
(Expected reference range 67 - 1134 cells/ μ l)		

Kindly correlate with clinical findings

***** End Of Report *****

Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology

